

ACADEMIC MEDICAL CENTER HEALTH SYSTEM

INPATIENT DISCHARGE SUMMARY

Department of Internal Medicine & Critical Care • 500 University Way • Tel: (555) 019-9000

Patient Name:	MONTGOMERY, ARTHUR C.	MRN / FIN:	903-882-14B / FIN-2026-9912
DOB / Age / Sex:	04/12/1954 (71 yrs) • Male	Admit / Discharge:	Sep 10, 2026 – Sep 24, 2026 (14 Days)
Attending Physician:	Dr. Eleanor Vance, MD (Pulmonology/CCM)	Primary Care MD:	Dr. Marcus Thorne, MD (Internal Med)
Admitting Service:	Medical Intensive Care Unit (MICU) / Stepdown	Discharge Location:	Subacute Rehabilitation Facility (SNF)

ALLERGIES & CLINICAL ALERTS

PENICILLIN: Severe (Anaphylaxis - hives, bronchospasm) | **CONTRAST DYE:** Moderate (Rash) | **FALL RISK:** High (Mobility deficit)

1. DISCHARGE DIAGNOSES

ICD-10 Coding Reference

Primary / Principal Discharge Diagnoses:

- Severe Community-Acquired Bacterial Pneumonia (Right Lower Lobe):** Complicated by septic shock and acute respiratory failure requiring invasive mechanical ventilation (Resolved). [ICD-10: J18.9, R65.21]
- Acute Kidney Injury (AKI) Stage 3:** Secondary to septic hypoperfusion and acute tubular necrosis, fully resolved to baseline renal function without renal replacement therapy. [ICD-10: N17.0]

Secondary Diagnoses & Comorbid Conditions Managed:

- Decompensated Type 2 Diabetes Mellitus:** Exacerbated by systemic stress and high-dose corticosteroid therapy. [ICD-10: E11.69]
- Atrial Fibrillation with Rapid Ventricular Response (RVR):** Paroxysmal, triggered by acute sepsis; converted to sinus rhythm. [ICD-10: I48.0]
- ICU-Acquired Muscular Weakness:** Secondary to prolonged critical illness and immobility. [ICD-10: G72.81]
- Essential Hypertension & Hyperlipidemia:** Baseline chronic medical conditions. [ICD-10: I10, E78.5]

2. PRESENTATION & ADMISSION NARRATIVE

Reason for Admission: The patient is a 71-year-old male with a history of COPD, hypertension, and T2DM who presented to the Emergency Department via EMS with a 3-day history of high fever (102.8°F), productive cough with purulent sputum, severe dyspnea, and altered mental status. Upon arrival, hemodynamics were unstable (BP 78/42 mmHg, HR 132 bpm, SpO2 82% on ambient air).

Initial ED Workup & Resuscitation: Arterial blood gas demonstrated severe hypoxemic respiratory acidosis (pH 7.21, pCO2 58, pO2 48). Chest CT confirmed extensive right lower lobe consolidation with parapneumonic effusion. Serum lactate was elevated at 4.8 mmol/L. The patient was endotracheally intubated for airway protection and placed on mechanical ventilation. Septic shock protocol was initiated with 3L crystalloid fluid boluses, continuous Norepinephrine infusion, and broad-spectrum empiric antibiotics (IV Vancomycin + Cefepime).

3. ADMISSION VS. DISCHARGE KEY DIAGNOSTICS

Laboratory / Diagnostic Parameter	Admission Value	Peak / Trough	Discharge Value	Reference Range
White Blood Cell Count (WBC)	22.4 x10 ³ / µL (H)	26.1 x10 ³ (Peak)	7.2 x10 ³ / µL	4.5 – 11.0 x10 ³
Serum Creatinine (AKI Marker)	2.85 mg/dL (H)	3.42 mg/dL (Peak)	1.05 mg/dL	0.70 – 1.30 mg/dL
Serum Lactate	4.8 mmol/L (H)	5.1 mmol/L (Peak)	1.1 mmol/L	0.5 – 2.2 mmol/L
Blood Glucose	18.5 mmol/L (H)	24.1 mmol/L (Peak)	6.1 mmol/L	< 6.1 mmol/L

4. DETAILED INPATIENT HOSPITAL COURSE

By Organ System

Pulmonary / Critical Care: Intubated on Day 1 for acute hypoxemic respiratory failure secondary to right RLL pneumonia. Sputum cultures grew *Streptococcus pneumoniae* (penicillin-resistant, cephalosporin-sensitive). Antibiotics were targeted to IV Ceftriaxone 2g daily on Day 3. The patient completed a 10-day total course of targeted antibiotic therapy. Lung protective ventilation strategies were utilized. A lung recruitment maneuver and diuresis enabled successful extubation on Hospital Day 6 to high-flow nasal cannula (HFNC), subsequently titrated down to room air by Day 10.

Cardiovascular & Hemodynamics: Septic shock required vasopressor support with Norepinephrine for 48 hours in the MICU. On Day 2, the patient developed paroxysmal Atrial Fibrillation with RVR (rates up to 150s), which was managed with a continuous Amiodarone infusion and IV Diltiazem. Reverted to normal sinus rhythm within 24 hours. Echo revealed an EF of 50-55% with mild LV hypertrophy and no valvular pathology. Transthoracic ECHO on Day 12 demonstrated preserved systolic function.

Renal & Metabolic: Acute Kidney Injury Stage 3 manifested with oliguria and elevated serum creatinine (peak 3.42 mg/dL). Managed conservatively with judicious hemodynamic optimization, avoidance of nephrotoxins, and fluid management. Renal function completely recovered to baseline creatinine (1.05 mg/dL) by Day 11 without requiring continuous renal replacement therapy (CRRT). Hyperglycemia was controlled using an intravenous insulin drip in the ICU, transitioned to a basal-bolus subcutaneous regimen (Glargine/Lispro) on the stepdown floor.

Physical Medicine & Rehabilitation: Due to prolonged bed rest and ICU stay, patient developed marked deconditioning and ICU-acquired weakness. Physical and Occupational Therapy evaluated the patient on Day 8. While cognitive function has returned to baseline, the patient requires moderate assistance (Mod-Asst) for transfers and ambulatory distance is limited to 40 feet with a rolling walker. Subacute rehabilitation was recommended for functional restoration prior to returning home.

5. KEY PROCEDURES & SPECIALIST CONSULTATIONS

Date	Procedure / Consultation	Specialty Service	Clinical Outcome / Findings
09/10/2026	Endotracheal Intubation & Central Line	Critical Care Medicine	Successful placement without complications. Line pulled Day 6.
09/11/2026	Diagnostic Thoracentesis	Interventional Pulmonology	450 mL turbid fluid removed. Exudative paraneoplastic rule-out.
09/12/2026	Transthoracic Echocardiogram (TTE)	Cardiology Service	EF 50-55%, mild diastolic dysfunction, no structural thrombus.
09/18/2026	Physical & Occupational Rehab Consult	Physiatry / PT / OT	Recommended 2-3 weeks SNF stay for mobility and ADL training.

6. OUTSTANDING & PENDING TEST RESULTS

1. Fungal Blood Culture (4-Week Hold): Final result pending (No growth at 14 days as of discharge).

2. Sputum Cytology Panel: Negative for malignancy; final immunohistochemistry stain pending.

Note: Attending MD Dr. Eleanor Vance will monitor pending results and contact the receiving SNF physician within 72 hours.

7. DISCHARGE MEDICATION RECONCILIATION

Complete Drug List

The following table reflects the complete reconciled medication regimen upon discharge. Changes from pre-admission medications are flagged.

Medication Name	Dose & Route	Frequency	Status	Clinical Indication / Special Instructions
Cefdinir	300 mg Oral	Every 12 hrs	NEW	Complete oral step-down for pneumonia (Finish 4 days).
Amiodarone	200 mg Oral	Once Daily	NEW	Maintenance rhythm control. Follow-up ECG in 2 weeks.
Apixaban (Eliquis)	5 mg Oral	Twice Daily	NEW	Stroke prevention secondary to new-onset Atrial Fibrillation.
Insulin Glargine (Lantus)	14 Units SubQ	At Bedtime	CHANGED	Dose increased due to glycemic instability (Pre-admit: 10U).
Insulin Lispro (Humalog)	Sliding Scale SubQ	With Meals	NEW	Correctional coverage for blood glucose > 180 mg/dL.
Lisinopril	10 mg Oral	Once Daily	CONTINUED	Hypertension. Hold if Systolic BP < 100 mmHg.
Metformin HCl	1,000 mg Oral	Twice Daily	STOPPED	Discontinued due to recent renal recovery. Re-evaluate in 4 wks.
Atorvastatin	40 mg Oral	Once Daily	CONTINUED	Hyperlipidemia management at bedtime.

8. TRANSITION OF CARE & REHABILITATION PLAN

- Discharge Disposition:** Transferred via non-emergent medical transport to Pinecrest Subacute Rehab Center.
- Physical & Occupational Therapy:** Daily aggressive physical therapy focusing on lower extremity strengthening, gait training, and transfer safety. Goal is unassisted ambulation prior to home transition.
- Dietary & Nursing Care:** 2,000 calorie diabetic diet. Fluid intake target 2.0L/day. Daily weight check. Monitor strict fingerstick blood glucose 4x daily (before meals and bedtime).
- Wound & Line Care:** Central line site in right subclavian area is clean, dry, intact (removed 09/16/2026). No active dressing needed.

9. FOLLOW-UP APPOINTMENTS

Healthcare Provider / Specialty	Facility / Clinic Address	Scheduled Date & Time
Dr. Eleanor Vance (Pulmonology)	AMC Outpatient Center, Suite 410	October 08, 2026 at 09:30 AM
Dr. Marcus Thorne (Primary Care)	Community Health Associates	October 12, 2026 at 02:15 PM
Outpatient Cardiology Clinic	Heart & Vascular Institute, Floor 2	October 20, 2026 at 11:00 AM

CRITICAL RED-FLAG WARNINGS & RE-ADMISSION PRECAUTIONS

The SNF clinical team or family must contact the attending physician or seek immediate emergency services (Call 911 / ED) if the patient develops:

- Fever above 101.0°F, shaking chills, or recurrent purulent sputum production.
- Acute onset of shortness of breath, chest pain, or oxygen saturation dropping below 92%.
- Signs of bleeding (e.g., severe bruising, dark tarry stools) secondary to new anticoagulant therapy.
- Unexplained rapid pulse (> 110 bpm) or severe confusion/lethargy.

